Hepatobiliary Case Study

A 35-year-old female presents to the ED with bilateral leg edema and increased abdominal distention. She has a past medical history of anxiety, depression, and hypertension. Family history is significant for nonalcoholic fatty liver disease. She is confused upon arrival to the ED, oriented to self and place only.

Assessment: Her abdomen is grossly distended and firm. 3+ pitting edema to bilateral lower extremities up to flank area. 2+ pitting edema to bilateral upper extremities. She is tachypneic with RR 27 but no accessory muscle use noted. She is able to speak 2-3 words between breaths. O2 sat 93% on 4L NC. BP 90/65.

1. What are the acute concerns for this patient?
2. What do you think is the cause of her symptoms based on assessment? What lab values do you expect to see?
3. What will you prep the patient for to relieve her symptoms? What medications do you anticipate giving her?

You draw the patient's labs with the following results. Explain the rationale for each of the lab values if not within normal range.

Na	133
K	4.7
CO2	18
BUN	55
Creatinine	2.22
Alkaline Phosphatase	150
ALT	42
AST	61
Protein	6.1
Bilirubin	4.9
PT	30.2
INR	2.9
Hgb	7.1
Platelet	24
Ammonia	90

How do you expect the patient's assessment to change if the interventions and treatment plan were completed?